

CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: TaylorPeriod: May 1 to June 1, 2026Report Date: June 2, 2026Coverage: 743/745h (100%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

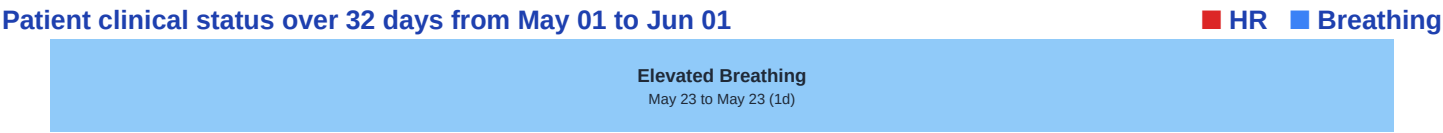
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	63 bpm	51 bpm	99 bpm
Breathing	16 brpm	12 brpm	30 brpm

Last 24 hours: no episodic events, vitals within normal range.

Episodic Events (last 24h)

No episodic events in the last 24 hours.

1 episodic events Detected, spanning 1h total. Conditions: Elevated Breathing. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
1	1h	Elevated Breathing	<1	100%

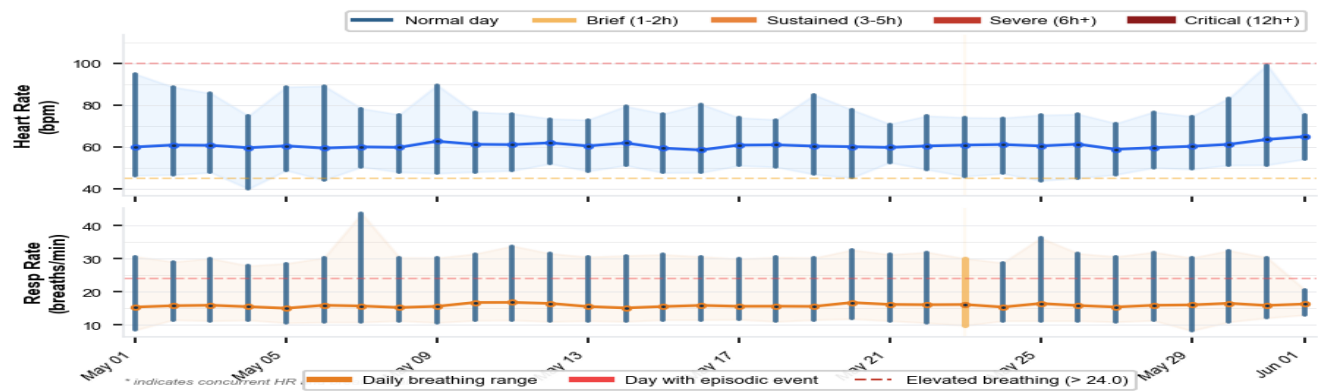
Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 23	8 AM to 9 AM	1h	1	Elevated Breathing	25	14	30	Check SpO2 and lung sounds; assess for fluid overload or early infection

Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

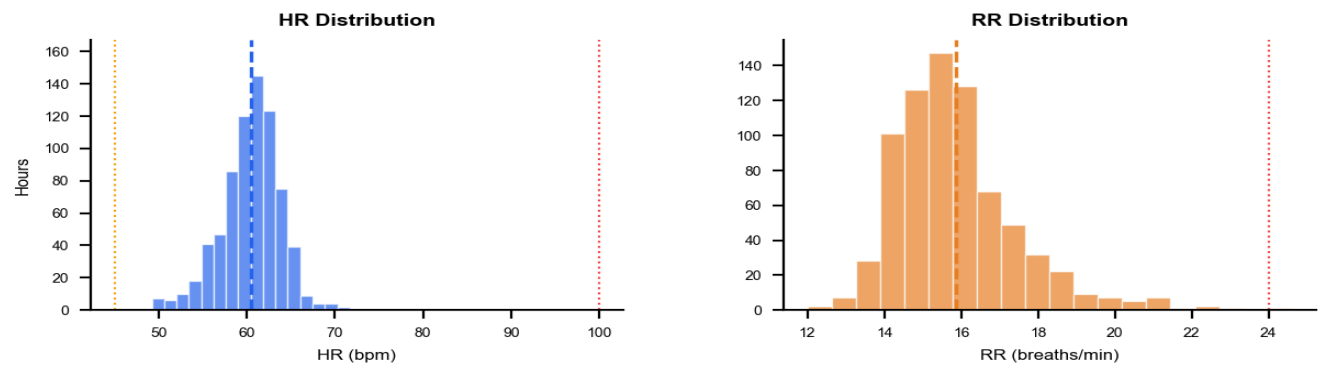
Suggested Clinical Review Actions

- Elevated Breathing (avg 25 brpm, peak 30 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

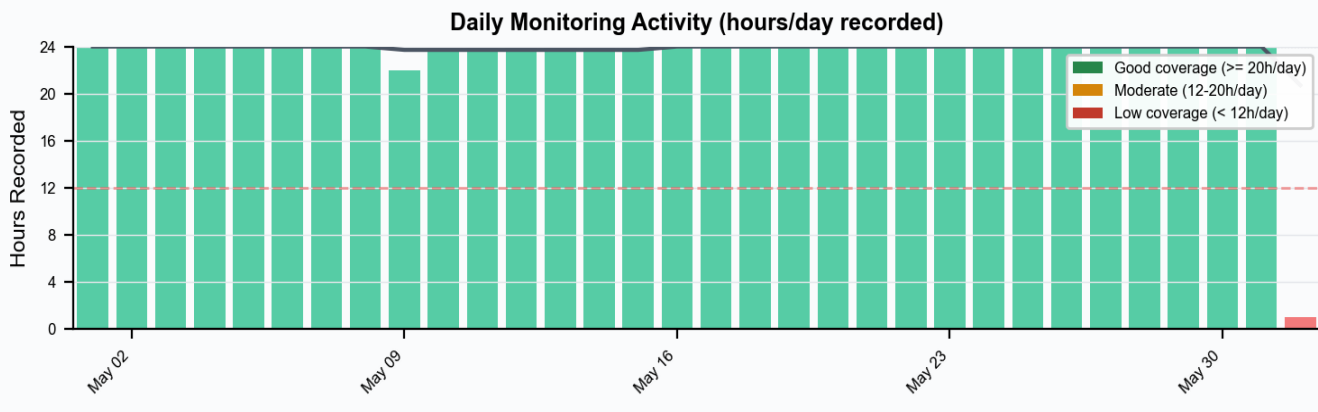


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.